Patient information

- Full name: Aziz Belghaieb
- Date of birth: 14 March 1999
- Sex: Male
- Patient ID: AB-2025-001
- Address: 12 Rue des Oliviers, 75019 Paris, France
- Phone: +33 6 12 34 56 78
- Emergency contact:
    - Name: Samir Belghaieb (brother)
    - Phone: +33 6 98 76 54 32

Visit details

- Facility: Centre Médical République
- Department: General Practice
- Attending physician: Dr. Marie Dupont, MD
- Visit date: 04 December 2025
- Visit type: Outpatient consultation
- Reason for visit (Chief complaint):
    - Intermittent chest discomfort and shortness of breath on exertion for 3 weeks.

History

- History of present illness:
    - Chest discomfort described as pressure in the left anterior chest, occurring mainly during fast walking or climbing stairs, lasting 5–10 minutes, relieved with rest, no radiation to arm or jaw, no syncope.
    - Shortness of breath with moderate exertion, no nocturnal dyspnea, no cough or fever.
- Past medical history:
    - Mild intermittent asthma diagnosed in childhood; last exacerbation 2 years ago, no hospitalizations.

    - No known history of hypertension, diabetes, or hyperlipidemia.
- Surgical history:
    - Appendectomy at age 15, uncomplicated.
- Medications:
    - Salbutamol inhaler 100 mcg PRN, approximately 1–2 times per week.
- Allergies:
    - No known drug allergies.
    - Mild seasonal allergic rhinitis to pollen.
- Family history:
    - Father: Myocardial infarction at 52 years, hypertension.
    - Mother: Type 2 diabetes diagnosed at 60 years.
    - No family history of sudden cardiac death or hereditary arrhythmias reported.
- Social history:
    - Non-smoker.
    - Alcohol: 1–2 beers on weekends.
    - Works as a software developer; mostly sedentary job.
    - Exercises irregularly (football with friends twice a month).

Review of systems (selected)

- Constitutional: No fever, chills, or recent weight loss.
- Cardiovascular: Exertional chest discomfort, no palpitations at rest, no edema reported.
- Respiratory: Exertional dyspnea; no chronic cough, no hemoptysis, no wheezing at rest.
- Gastrointestinal: No nausea, vomiting, or abdominal pain.
- Neurological: No syncope, seizures, or focal deficits.

Physical examination

- Vital signs:

    - Blood pressure: 128/78 mmHg
    - Heart rate: 84 bpm, regular
    - Respiratory rate: 16/min
    - Temperature: 36.7 °C
    - Oxygen saturation: 98% on room air
- General: Alert, oriented, no acute distress, normal body habitus.
- Cardiovascular: Normal S1/S2, no murmurs, rubs, or gallops; peripheral pulses 2+ and symmetric; no peripheral edema.
- Respiratory: Clear to auscultation bilaterally; no wheezes, rales, or rhonchi.
- Abdomen: Soft, non-tender, no organomegaly.
- Extremities: No clubbing, cyanosis, or edema.
- Neurological: Grossly intact; normal gait and speech.

Investigations

- Electrocardiogram (ECG):
    - Normal sinus rhythm, rate 80 bpm; no ST-segment changes or pathological Q waves.
- Laboratory tests ordered:
    - Complete blood count (CBC) – within normal limits.
    - Basic metabolic panel – within normal limits.
    - Fasting lipid profile – pending at time of note.
- Additional testing:
    - Exercise stress test scheduled within 2 weeks to evaluate exertional symptoms given family history of premature coronary artery disease.

Assessment

- 26-year-old male with intermittent exertional chest discomfort and dyspnea, normal examination and resting ECG, with family history of premature coronary artery disease.
- Differential diagnosis:
    - Low-to-intermediate risk stable angina.

    - Musculoskeletal chest pain related to deconditioning or posture.
    - Exercise-induced bronchospasm (given history of asthma).

Plan

- Diagnostic:
    - Proceed with exercise stress test.
    - Obtain fasting lipid profile and HbA1c to screen for cardiovascular risk factors.
- Treatment and recommendations:
    - Advise gradual increase in aerobic exercise (30 minutes, 5 days per week) as tolerated, avoiding high-intensity exertion until stress test completed.
    - Educate patient about warning signs (rest pain, prolonged chest pain, radiation, syncope) requiring immediate emergency evaluation.
    - Continue salbutamol inhaler PRN; review inhaler technique.
    - Counsel on heart-healthy diet and maintaining normal weight.
- Follow-up:
    - Follow-up visit in 3 weeks or sooner if symptoms worsen.
    - Review stress test and lab results at that visit.
- Physician signature:
    - Dr. Marie Dupont, MD
    - Date/time: 04 December 2025, 15:40